

العمر(تاريخ الميلاد) : 1965-06-22 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

----- التعليق على التشخيص :

GR1-E

رستم خیل

لدى حسين رمضان

صحة الايام العرفقة
للجنة مدير

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناقشة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

ملاحظات :

المجلس الطبي المتخصص

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Name: سوسن محمد محمود عوض National ID: 26506222100088	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



موسى
محمد محمود عوض
الشواشنة
مركز يوسف الصديق - الفيوم

١٩٩٥/٠٧/٢٢ ٢٦٥.٦٢٢٢١.٠٠٠.٨٨
IB8711204

٢٠١٩/٠٧ ٢٦٥.٦٢٢٢١.٠٠٠.٨٨
حاصله على دبلوم تجاره



أنثى مسلمة أرملة

البطاقة سارية حتى ٢٠٢٦/٠٧/٠٧ ٦





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

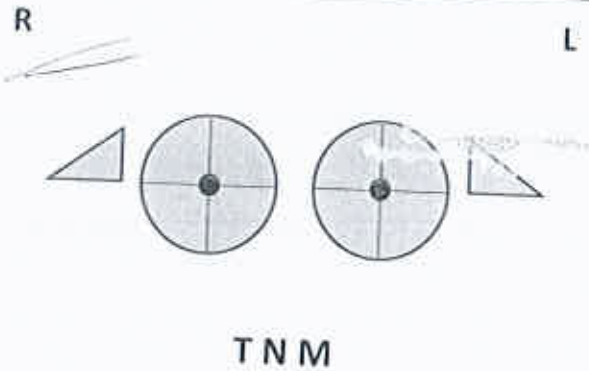
27 / 11 / 2022

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Name	سوس محمد محمد	MRN	٢٩٩٧ / ٢٩٩٧
Age	56	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: ID 19 II			
ER: 4/8	PR: 8/8	HER 2: 3+	Ki67:
Sonomammography:			
Mets Workup:			



Recommendations: 51P 6 cycles TCHP, & WLE & ALND
21/6/22 → ypT0N1/2 with ECE → 4 cycles
of adj. TDM-1 & PORT

MDT OUTCOME & PLAN

- Continue TDM-1
- Hormonal therapy with AI

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. إسلام أبو د. نرسيه					



Patient Name	: الأستاذ / سوسن محمد محمود عوض	Request No.	: 130930
Patient No.	: 26506222100088	Request Date	: 07/12/2021
Path No.	: SR-702/21	Verification Date	: 09/12/2021
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referring Doc :			

Pathology Report

Clinical Data:
Right breast cancer.

Nature of the specimen:
Slide revision.

Gross Examination:
One paraffin block (8705/21) and 3 slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II
- Angioinvasion: Negative
- Ductal carcinoma in situ: Negative
- Examination of ER stained slide show intense nuclear staining in 90% tumor cells.
- Examination of PR stained slide show moderate nuclear staining in 80% of tumor cells.
- Examination of HER2 stained slide show intense staining in >10% of tumor cells with complete membranous staining.

Diagnosis:
Right breast cancer; Slide revision:
- Invasive duct carcinoma, grade II
• Biomarkers:

- o ER: Allred score 8/8
- o PR: Allred score 6/8
- o Her2: Positive (score 3+)

Prof. Dr. Aml Hareedy
Dr. Walaa Ghonaim

Patient Name : الأستاذ / مومن محمد محمود عزمي
 Patient No. : 26506222100088
 Path No. : 1733/2022
 Sex : Female
 Age : 56 Y
 Referring Doc :

Request No. : 13002160
 Request Date : 21/06/2022
 Verification Date : 05/07/2022
 Financial Category : أورام دار السلام - نفقة الدولة
 Department : Out Patient
 Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Right breast mass, post NACT.

Nature of the specimen: Right wired WLE specimen and Right axillary lymph node clearance.

Gross Examination: Two specimens were received:

1- Procedure: Right wired WLE specimen: measured 5.5x4.5x1cm with skin covering measured 3.5x1.5cm.

• Tumor extent

o Number: Cut section revealed ill defined grayish white rubbery tumour bed site.

o Size: measured 5.5x4.5x1cm.

o Skin involvement: Nipple and areola were unremarkable grossly.

• Distance from margins: Away 0.5cm from lateral margin (least), 1 cm from skin and 2 cm from deep margin.

2- Axillary clearance:

Fibrofatty tissue piece measured 10x5.5x2.5cm entangling 21 lymph nodes up to 1.5x1cm.

• Frozen section: Free resection margins.

Microscopic examination:

• Histological type: No viable tumour cells in previously diagnosed invasive carcinoma.

• Angiomyosin: No.

• Ductal carcinoma in situ: No.

• Lymph node involvement:

Positive in (1/21) with extracapsular extension.

Two lymph nodes showed histiocytes and fibrosis.

• Resection Margins: Free resection margins.

Verified by :

DR.Menna Nabil

Printing Date : 18/07/2022 11:25:37

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Printing By : DR.Menna Nabil

Patient Name : الأستاذ / مومن محمد محمود عوض
Patient No. : 26506222100088
Path No. : 1733/2022
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13002160
Request Date : 21/06/2022
Verification Date : 05/07/2022
Financial Category : نفقة الدولة - إرام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

• Histologic response to therapy: ~~Complete~~ pathological response.

Diagnosis:

Right breast mass; Right wired WLE specimen and Right axillary lymph node clearance:

- No viable tumour cells in previously diagnosed
- Complete pathological response to the therapy.
- No Angioinvasion.
- No Ductal carcinoma in situ.
- Lymph nodes were Positive in (1/21) with extracapsular extension.
- Free resection margins.

Prof. Dr. Gamal Abdellatty.
Dr. Menna Nabil.

Verified by :
DR. Menna Nabil

Printing Date : 18/07/2022 11:25:37

Page 2 of 2

Printing By : DR. Menna Nabil

PATIENT NAME: SOSN MOHAMMED MOHMOUD AWAD	PATIENT ID #: 2021171459
GENDER: Female	ORDER #: 1315551
AGE: 56 y	DATE OF SERVICE: 14-Nov-2021 11:07 AM

Report:

99mTc-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

FINDINGS:-

Degenerative changes at dorso- lumbar vertebrae.

Arthritic changes at knees.

Both kidneys are seen.

Conclusion:-

No definite evidence of metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed

15-Nov-2021 01:11 PM

PATIENT: SOSN MOHAMMED MOHMOUD AWAD, 2021171459, Female, 56 y

Report version 1, 17-Nov-2021 11:03 AM

Page 1 of 1

PATIENT NAME: SOSN MOHAMMED MOHMOUD AWAD	PATIENT ID #: 2021171459
GENDER: Female	ORDER #: 1315559,1315560
AGE: 56 y	DATE OF SERVICE: 13-Nov-2021 04:20 PM

Report: - all studies

Multi-slice post contrast CT scan of the chest, abdomen and pelvis revealed:

(I) Chest:

- The right breast shows an enhancing mass lesions and nodules in the lower outer quadrant, the largest measures about 3.9 x 2.5 cm in its maximum axial dimensions with parenchymal edematous changes and skin thickening.
- Multiple enlarged right axillary lymph nodes are seen in levels I, II and III, the largest measures about 2 x 1.3 cm in its maximum axial dimensions.
- Prominent right internal mammary lymph nodes are seen, the largest is seen measuring about 12 x 7 mm in its maximum axial dimensions.
- Prominent infraclavicular LNs are seen the largest measures about 11 x 13 mm.
- No pulmonary parenchymal nodules, cavitory lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is mildly enlarged in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

- The axial and appendicular skeleton shows:
 1. Dorsolumbar spondylo-degenerative changes.
 2. A small left acetabular sclerotic focus, likely bone island.

OPINION:

- *Right breast mass lesions and nodules*
- *Right axillary lymphadenopathy in levels I, II and III, as well as prominent right internal mammary lymph nodes.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Reported By,

Dr. Ahmed Abdelmoneim, MSc

Dr. Mahmoud Saleh, MD

14-Nov-2021 01:46 PM

- The axial and appendicular skeleton shows:
- 1. Dorsolumbar spondylo-degenerative changes.
- 2. A small left acetabular sclerotic focus, likely bone island.

OPINION:

- *Right breast mass lesions and nodules*
- *Right axillary lymphadenopathy in levels I, II and III, as well as prominent right internal mammary lymph nodes.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Reported By,

Dr. Ahmed Abdelmoneim, MSc
Dr. Mahmoud Saleh, MD 14-Nov-2021 01:46 PM

Patient's Name: Sawsan Mohammed Mahmoud Awad
Date: 2026-08-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.
ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of right breast cancer
- Right upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

I- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.
- ❖ right upper lobar subpleural reticulations and fibrotic strands likely post therapeutic sequel

Dr.Assem Ahmed